

MEDILINK | THE LEGACY TRIAD

Revitalized championship handbook • Student instructions • Story architecture • Competition operations • Analytic scoring

One school. Four delegates. Three arenas. A reputation earned under pressure.

October 2026 | Proposed replacement framework

This handbook replaces the former five-event catalogue and two-group delegation model. It establishes three flagship Legacy events: The Sovereign Ledger, Nightfall: Code Meridian, and The Janus Protocol. Regionals unfold within one competition day. State scheduling expands the experience, but duration is not the event's identity.

The defining experience is a chain of consequential decisions: read uncertain evidence, commit resources, meet rivals, absorb a reversal, revise responsibly, and face public accountability. Teams cannot win by delivering one polished presentation. Their work must survive execution, cross-examination and changing facts.

Adoption status: detailed design proposal, not an already ratified rulebook or field-tested guarantee. Fictional figures illustrate case construction; final cases, dates and sites require approval and piloting. The scoring and logistics recommendations are explicitly identified where they extend the user's established requirements.

1 | Constitution and qualification

One chapter delegation

- Each school chapter registers exactly one team of four high-school members. The same four constitute the chapter's entire Legacy delegation for the season. No second group, extra specialist or alternating event roster.
- Teams may enter one, two or all three Legacy events. Qualification is separate for each event; a chapter may qualify in several or none.
- The roster locks before Regionals. A nationally approved replacement is permitted only for documented withdrawal or serious unavailability before a later round, not to acquire a specialist. No mid-event substitution.
- Each student owns a functional role and must understand cross-role decisions. Rotate leadership between phases if useful, but log ownership changes.
- Legacy delegates may participate in regular events under the regular-event rules. Legacy scheduling must not assume the four students can compete in simultaneous events.

Round	Binding pathway
Regionals	Top three eligible teams from each regional pool, separately in each event, advance to States. Ranking uses Regional raw score.
States	All qualifying regional teams within the state compete. The single highest cumulative Regional–State score in each event advances to Nationals.
Nationals	State champions compete for one national title in each event. National duration and final season formula require published adoption.

Example: a state with four regional pools can have twelve State entries per event, not three. A pool with fewer than three eligible teams advances its eligible teams; no artificial empty qualifiers. Publish pool boundaries, capacity and eligibility before registration.

Cumulative improvement formula — proposed

Each round is scored out of 1,000. Recommended State standing = 35% Regional raw score + 65% State raw score. This rewards growth without erasing Regionals. Use performance points, not placement bonuses, because raw quality should survive a competitive regional field.

Chapter	Regional	State	Cumulative
Ashford	920	800	842.0
Belmont	830	920	888.5
Cedar	780	950	890.5

Cedar advances despite the lowest Regional score in this example. State standing is not the same as State-only placement. Calculate on unrounded values; display one decimal. Proposed National standing = 15% Regional + 25% State + 60% National. Publish or replace this national proposal before registration; never change weights after seeing results.

2 | Fair scoring across a season

Carryover depends on comparable Regionals

- Within each state, use the same secured case, core injects, tools, timing, actor scripts, scoring criteria and constraint model across regional pools. Protect materials when sites compete at different times.
- Use three independently scoring judges where feasible. Average criterion scores after resolving arithmetic or rule errors. Calibrate panels with shared exemplar work before competition.
- Do not use harder cases at one regional and carry raw scores as if comparable. If case equality cannot be maintained, publish a validated alternative before the season rather than inventing post-hoc adjustments.
- State ties: higher State raw score; then State safety/clinical criterion; then live-adaptation criterion; then a common 15-minute tie case. National ties use National raw score followed by the equivalent criteria.
- If an integrity penalty or timing error affects carryover, correct both raw and cumulative results before certification.

What rivalry may and may not decide

Rival encounters test negotiation, evidence and composure. Teams earn points for defensible decisions, concessions, challenges and responses—not simply for beating a weaker opponent. An uncooperative rival cannot deny a team all transaction points. An actor fallback provides an equivalent chance to demonstrate skill when necessary.

Three distinct chapter identities

Arena	What a strong chapter becomes known for
The Sovereign Ledger	Disciplined capital strategy, financial nerve and protection of essential care.
Nightfall: Code Meridian	Command discipline, safe logistics and leadership during cascading failure.
The Janus Protocol	Technical skepticism, clinical validation and accountable innovation.

Optional chapter histories can record titles, verified comeback margins, returning matchups and respectful encounter recaps. These are recognition mechanisms, not extra qualification points. Obtain consent for public student images and quotes. Keep confidential individual notes out of rivalry publicity.

3 | Competition-day architecture

Regional schedule — one event, one day

Time	Controlled phase
08:00–10:00	Full case release; supervised reading, analysis and role allocation.
10:00–10:45	Opening decision and strategy lock.
10:45–11:45	First execution and rival encounter.
11:45–12:30	Protected lunch.
12:30–13:30	Consequence inject and revised decisions.
13:30–14:30	Major reversal and controlled replanning.
14:30–15:30	Final hearing and individual questioning.
15:30–16:00	After-action submission and artifact lock.
16:00 onward	Score audit, provisional results and appeal window.

This schedule is a proposed operating template. Parallel rooms provide each team equal hearing time. Include transition buffers and accessibility accommodations. All five storyline acts fit within the Regional day by reducing packet volume and transaction count—not by removing meaningful adaptation.

Expanded State logistics

States may distribute opening analysis and early encounters on the first day, followed by the major reversal, final execution and accountability on the next. Put exact reporting, breaks and release times in the event notice. Do not promote “multi-day” as the event’s main feature. No overnight homework, midnight injects or sleep-deprivation tactics.

Lock case notes and digital artifacts overnight; return them at the next controlled session. Students may contact family normally, but outside assistance on the active case remains prohibited. The same four-person team requires non-overlapping blocks for all three events at every level. Publish travel and participation costs early.

Advance preparation

Recommended release: broad topic, skills blueprint, sample templates, formula sheet and allowed-tool list eight weeks before Regionals. The complete fictional case releases only at monitored check-in. Provide offline spreadsheets, paper and calculators consistently. No open internet, generative AI, personal messaging or outside coaches during controlled work unless a specific rule expressly authorizes a tool.

THE SOVEREIGN LEDGER

Medicine × Finance | One four-student chapter team | Five-act live championship

Opening story

Asterion Health Alliance serves a metropolitan center and three community hospitals. Its unrestricted capital envelope is \$48 million, with a \$6 million minimum reserve. Competing investments exceed available funds. A rival provider is capturing profitable outpatient demand, a maternity pathway needs safer transfer capacity, and a cyber infrastructure replacement cannot be delayed indefinitely. A lender wants stability; clinicians want equipment; communities want access. The team must decide what deserves capital and what it will refuse.

The four-person operating model

- Finance strategist: sources/uses, debt, cash flow and downside cases.
- Clinical access officer: safety floors, service continuity, equity and transfer feasibility.
- Investment/risk analyst: sensitivities, workforce dependencies and implementation.
- Negotiation/board lead: partner terms, concessions, public defense and accountability.

Log role assignments at check-in. Students may rotate responsibility between acts, but every owner must explain their decisions. Judges ask cross-role questions because a finance choice can change safety, a technical choice can change staffing, and a management decision can change liquidity.

The secured case packet

- Income statement, balance sheet, cash forecast and debt covenant schedule.
- Service-line volume, margin, staffing, payer mix, quality and travel-time/access maps.
- Eight project dossiers with cost, recurring expense, capacity dependencies and clinical impact.
- Partner bids, financing choices, acquisition restrictions and minimum safety/service obligations.
- A quantified no-investment baseline and an assumptions worksheet.

Each exhibit carries an ID, timestamp, units and reliability status. Distinguish confirmed facts, estimates, allegations and binding constraints. A contradictory actor statement is not automatically true. Teams maintain an assumptions register and cite numbered evidence.

THE SOVEREIGN LEDGER | The five-act storyline

Act I: The Opening Ledger

Lock a five-year portfolio, reserve policy and clinical-access floor. Explain rejected investments; do not claim every project will pay for itself.

Required output: Capital allocation sheet, cash schedule, evidence index and assumptions register.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act II: The Exchange

Negotiate a shared-service agreement with a rival hospital team. A bounded sealed-bid procurement exercise allocates fictional vendor capacity. Safety-critical care itself is never auctioned.

Required output: Signed term sheet or justified walk-away, risk allocation and alternative plan.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act III: The Covenant Breach

A common payer update lowers expected receipts. The consequence engine applies previously stated liquidity and covenant relationships. High leverage is not automatically wrong, but becomes costly.

Required output: Revised cash forecast, covenant test, project sequencing and access mitigation.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act IV: The Benefactor's Clause

A donation appears with restrictive service commitments. A second exhibit reveals that the favored growth project creates a staffing bottleneck. Accept, counteroffer or reject with evidence.

Required output: Donation decision, revised staffing/capital plan and downside comparison.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act V: The Sovereign Board

Clinician, lender, labor and community actors challenge the final portfolio. A rival files one exhibit-based challenge. Close with first-100-day execution and accountability.

Required output: Final board book, three-scenario model, ownership milestones and public explanation.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

THE SOVEREIGN LEDGER | Strategy and rival encounters

Several legitimate routes to victory

- Phased expansion with liquidity preserved.
- Shared infrastructure instead of duplicate purchases.
- A lower-margin portfolio justified by essential access and credible financing.
- Partnerships with explicit service-quality and exit conditions.

Novelty is not automatically superior. Judges reward feasible alternatives supported by the record. A team can win without accepting a deal, deploying a product or expanding services, provided its alternative meets constraints and withstands challenge.

Hard constraint checks

- Capital available after reserve: \$42 million; reserved cash cannot also fund projects.
- Recurring operating expense must be financed separately from one-time capital.
- Every signed deal has an owner, funding source and contingency.
- A donation is evaluated for long-term obligations, not its headline amount.

A fair confrontation format

Use pods of three or four and rotate counterparties. Publish seeding criteria before competition. An odd field uses a trained actor counterpart. Each team receives one formal rival challenge in designated phases: a numbered exhibit, claimed implication and question. The recipient gets a fixed response window. Judges score the challenge and response independently; no point stealing.

Negotiation has four recorded steps: opening offer; counteroffer with rationale; concession or walk-away; final terms and fallback. Private agreements outside the monitored encounter are invalid. A rival's refusal cannot eliminate the chance to earn analytic points: teams defend their proposed terms and alternative to the panel.

Consequence engine

Common injects arrive at equal logical checkpoints. Conditional consequences follow disclosed rule categories such as liquidity limits, staffed capacity or validation gaps. Publish the model's constraint relationships and bounded effects in the case instructions, not a secret random multiplier. Staff verify the engine before releasing consequences. Scenario outcomes do not automatically prove decision quality; score controllable analysis and execution.

The drama must be earned

A first twist changes the environment. A second challenges an assumption. The final act tests recovery and accountability. Do not reverse binding rules for surprise, invent evidence without sufficient response time, or make the winning answer depend on guessing the writer's preference. Rivalry grows from consequential decisions and earned rematches, not harassment or simulated patient deaths as spectacle.

THE SOVEREIGN LEDGER | 1,000-point scoring guide

For each criterion, judge a 0–4 anchor level. Awarded points = maximum × level ÷ 4. Half-levels require a written rationale. Each criterion is scored once over the full evidence trail; phase feedback is not additional points. Three judges independently rate before arithmetic averaging.

Criterion / maximum	Evidence for high scores; failure pattern
Financial integrity and capital allocation / 220	Balanced sources and uses, correct capital/operating treatment, reserves, covenants and realistic financing. Failure pattern: Double-counting funds, invented grants or omitted recurring costs.
Clinical access, safety and equity / 200	Meets safety floors, identifies access effects and offers feasible continuity alternatives. Failure pattern: Margin-only decisions, unsupported transfer assumptions or unsafe closures.
Evidence and sensitivity analysis / 150	Tests volume, price, staffing and implementation assumptions using numbered exhibits. Failure pattern: False precision or unexamined forecasts.
Transactions and negotiation / 120	Defensible concessions, enforceable terms, clear value and an independent fallback. Failure pattern: Agreement at any cost or no explanation of rejected terms.
Adaptation and strategic coherence / 160	Updates finance, access and execution together after each reversal. Failure pattern: Cosmetic changes, concealed losses or incompatible plans.
Implementation and board defense / 100	Named owners, milestones, risks and clear answers to stakeholder concerns. Failure pattern: A portfolio with no feasible delivery plan.
Individual mastery / 50	Each student explains one calculation and one cross-role consequence. Failure pattern: One analyst carries the whole team.

Shared performance anchors

Level	Standard
4 — Exceptional	Accurate, complete, case-specific and feasible; resolves conflicts, meets constraints and adapts with explicit reasoning.
3 — Strong	Sound core strategy with minor gaps; supported evidence and credible implementation.
2 — Developing	Partly sound but material assumption, calculation or delivery gaps remain.
1 — Weak	Generic, poorly evidenced or substantially flawed; revisions do not repair the core issue.
0 — Absent/invalid	Required evidence absent, fabricated, irrelevant or incompatible with binding constraints.

THE SOVEREIGN LEDGER | Scoring worksheet and progression

Criterion	Max	Level	Award
Financial integrity and capital allocation	220	_____	_____
Clinical access, safety and equity	200	_____	_____
Evidence and sensitivity analysis	150	_____	_____
Transactions and negotiation	120	_____	_____
Adaptation and strategic coherence	160	_____	_____
Implementation and board defense	100	_____	_____
Individual mastery	50	_____	_____
TOTAL	1000		_____

Individual defense

Ask every student two equivalent short questions: one on work they owned and one on a cross-role dependency. Ask for evidence and reasoning, not obscure trivia. Individual mastery contributes only its listed points; no hidden bonus can change chapter qualification. Record role-specific feedback for development.

Round-specific complexity

Round	Depth, not duration, defines challenge
Regional	One-day five-act case with a concise evidence packet, one principal rival encounter and common core injects.
State	Richer data, tighter constraints, deeper stakeholder conflict and more developed encounter cycles. Expanded logistics appear in the event notice.
National	System-level dependencies and harder uncertainty, with the same published criterion weights. Scheduling approved separately.

What separates this from regular events

A sequence of locked actions creates consequences. Rival encounters test judgment. Technical and clinical evidence must survive execution. The final hearing examines both what the team chose and how it revised. No slide deck can replace the work history.

Safety boundary

All cases are fictional. No patient care, medication administration, real clinical records, unapproved human testing, legal representation or actual investment solicitation. Unsafe proposals affect the relevant scoring criterion; deliberate misconduct follows a separate review process. Do not automatically disqualify a student for an honest analytical mistake.

NIGHTFALL: CODE MERIDIAN

Medicine × Management | One four-student chapter team | Five-act live championship

Opening story

Meridian Regional is a fictional 320-bed referral hospital. An electronic-record outage coincides with a storm that limits transport and staff arrivals. Emergency demand rises while neighboring hospitals strain. Oxygen, blood and qualified coverage are finite. Elective procedures support cash flow but compete for capacity. The team must hold the organization together without making unsafe promises.

The four-person operating model

- Incident commander: authority, objectives, decision cadence and logs.
- Clinical continuity lead: service priorities, handoffs, escalation and transfer safety.
- Workforce/logistics chief: staffed beds, credentials, rest limits, inventory and transport.
- Finance/public trust lead: spending authority, cash exposure, accurate messages and board oversight.

Log role assignments at check-in. Students may rotate responsibility between acts, but every owner must explain their decisions. Judges ask cross-role questions because a finance choice can change safety, a technical choice can change staffing, and a management decision can change liquidity.

The secured case packet

- Census, acuity bands, staffed-bed map, procedure schedule and transfer capacities.
- Staff credentials, coverage minima, absence forecast and fatigue constraints.
- Inventory, lead times, oxygen/blood limits and downtime workflow options.
- Cash buffer, emergency authority, interruption costs and revenue assumptions.
- Stakeholder messages and an explicit command authority matrix.

Each exhibit carries an ID, timestamp, units and reliability status. Distinguish confirmed facts, estimates, allegations and binding constraints. A contradictory actor statement is not automatically true. Teams maintain an assumptions register and cite numbered evidence.

NIGHTFALL: CODE MERIDIAN | The five-act storyline

Act I: The First Thirty Minutes

Establish command and immediate priorities. Define what can continue safely, what must change and who can authorize action.

Required output: Command chart, first-30-minute actions and service-priority matrix.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act II: The Mutual Aid Table

Negotiate with teams representing neighboring hospitals. Transfer capacity, staff assistance and supply support require safe handoffs and realistic timing.

Required output: Mutual-aid agreement, transfer log, balanced resource board and fallback.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act III: The Cascade

A road closure interrupts resupply. A medication-reconciliation warning exposes a weakness in downtime processes. Prior decisions affect queues through disclosed relationships.

Required output: Revised resource/staff plan, verification checkpoint and internal/public update.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act IV: The False All-Clear

Some systems return, but records remain inconsistent. Staff fatigue worsens while a public report incorrectly claims full restoration. Reopening needs verification, not optimism.

Required output: Restore/hold decision, verification criteria, rest plan and correction statement.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act V: The Meridian Inquiry

Board, regulator, staff representative and patient actors demand explanation. A rival challenges one allocation. Show restoration criteria, financial mitigation and lessons.

Required output: 24-/72-hour plans, exposure report, recovery thresholds and after-action reforms.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

NIGHTFALL: CODE MERIDIAN | Strategy and rival encounters

Several legitimate routes to victory

- Staged service restoration with verification gates.
- Distributed stabilization partnerships and explicit handoffs.
- Conservative capacity decisions paired with credible mutual aid.
- Controlled downtime processes that lower workload without weakening safety.

Novelty is not automatically superior. Judges reward feasible alternatives supported by the record. A team can win without accepting a deal, deploying a product or expanding services, provided its alternative meets constraints and withstands challenge.

Hard constraint checks

- An empty bed is not a staffed bed.
- Transport time and receiving-site acceptance must be confirmed in the fictional record.
- Supplies and staff cannot serve two places simultaneously.
- Restoration requires technical and clinical verification.

A fair confrontation format

Use pods of three or four and rotate counterparties. Publish seeding criteria before competition. An odd field uses a trained actor counterpart. Each team receives one formal rival challenge in designated phases: a numbered exhibit, claimed implication and question. The recipient gets a fixed response window. Judges score the challenge and response independently; no point stealing.

Negotiation has four recorded steps: opening offer; counteroffer with rationale; concession or walk-away; final terms and fallback. Private agreements outside the monitored encounter are invalid. A rival's refusal cannot eliminate the chance to earn analytic points: teams defend their proposed terms and alternative to the panel.

Consequence engine

Common injects arrive at equal logical checkpoints. Conditional consequences follow disclosed rule categories such as liquidity limits, staffed capacity or validation gaps. Publish the model's constraint relationships and bounded effects in the case instructions, not a secret random multiplier. Staff verify the engine before releasing consequences. Scenario outcomes do not automatically prove decision quality; score controllable analysis and execution.

The drama must be earned

A first twist changes the environment. A second challenges an assumption. The final act tests recovery and accountability. Do not reverse binding rules for surprise, invent evidence without sufficient response time, or make the winning answer depend on guessing the writer's preference. Rivalry grows from consequential decisions and earned rematches, not harassment or simulated patient deaths as spectacle.

NIGHTFALL: CODE MERIDIAN | 1,000-point scoring guide

For each criterion, judge a 0–4 anchor level. Awarded points = maximum × level ÷ 4. Half-levels require a written rationale. Each criterion is scored once over the full evidence trail; phase feedback is not additional points. Three judges independently rate before arithmetic averaging.

Criterion / maximum	Evidence for high scores; failure pattern
Patient safety and continuity / 250	Safe priorities, verified handoffs, escalation and decisions within credential/capacity limits. Failure pattern: Unsafe diversion, unverified medication workflow or fictitious capacity.
Command and decision discipline / 150	Clear authority, contemporaneous logs, objectives and owners. Failure pattern: Contradictory orders or retroactive rewriting.
Resource and workforce execution / 180	Balanced assets, safe rest, feasible transport and documented mutual aid. Failure pattern: Double allocation, impossible coverage or ignored fatigue.
Adaptation and restoration / 170	Operational revisions follow new evidence; restoration meets defined checks. Failure pattern: Premature reopening or rigid adherence to the first plan.
Financial resilience / 100	Transparent costs, revenue exposure, reserves and justified emergency expenditure. Failure pattern: Safety sacrificed for revenue or costs ignored.
Trust and stakeholder communication / 100	Accurate accessible updates, uncertainty labels and timely correction. Failure pattern: False reassurance or concealed risk.
Individual command mastery / 50	Each student explains their actions and another role's dependency. Failure pattern: One commander answers for everyone.

Shared performance anchors

Level	Standard
4 — Exceptional	Accurate, complete, case-specific and feasible; resolves conflicts, meets constraints and adapts with explicit reasoning.
3 — Strong	Sound core strategy with minor gaps; supported evidence and credible implementation.
2 — Developing	Partly sound but material assumption, calculation or delivery gaps remain.
1 — Weak	Generic, poorly evidenced or substantially flawed; revisions do not repair the core issue.
0 — Absent/invalid	Required evidence absent, fabricated, irrelevant or incompatible with binding constraints.

NIGHTFALL: CODE MERIDIAN | Scoring worksheet and progression

Criterion	Max	Level	Award
Patient safety and continuity	250	_____	_____
Command and decision discipline	150	_____	_____
Resource and workforce execution	180	_____	_____
Adaptation and restoration	170	_____	_____
Financial resilience	100	_____	_____
Trust and stakeholder communication	100	_____	_____
Individual command mastery	50	_____	_____
TOTAL	1000		_____

Individual defense

Ask every student two equivalent short questions: one on work they owned and one on a cross-role dependency. Ask for evidence and reasoning, not obscure trivia. Individual mastery contributes only its listed points; no hidden bonus can change chapter qualification. Record role-specific feedback for development.

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THE JANUS PROTOCOL

Medicine × Technology | One four-student chapter team | Five-act live championship

Opening story

A fictional hospital consortium is considering JANUS, an AI deterioration-prediction and care-routing platform. Aggregate performance looks compelling and projected savings are large. Local data are fragmented; subgroup estimates are uncertain; the contract restricts audit access. A competing product offers less automation but better integration. The team must distinguish an impressive demo from an accountable clinical system.

The four-person operating model

- Clinical validation lead: outcome relevance, error consequences and safety case.
- Data/technical lead: denominators, missingness, leakage, calibration and local validity.
- Governance/security lead: privacy, audit access, incident ownership and shutdown authority.
- Implementation/finance lead: workflow, total cost, procurement, staffing and alternatives.

Log role assignments at check-in. Students may rotate responsibility between acts, but every owner must explain their decisions. Judges ask cross-role questions because a finance choice can change safety, a technical choice can change staffing, and a management decision can change liquidity.

The secured case packet

- Cohort tables, confusion matrices, prevalence, subgroup counts and uncertainty information.
- Data dictionary, provenance, missingness, train/test separation and local pilot results.
- Interface/workflow maps, clinician feedback, alert burden and fallback options.
- Vendor contracts, interoperability/data rights, total-cost assumptions and competing solutions.
- Incident log, acceptance-threshold form and deployment restriction template.

Each exhibit carries an ID, timestamp, units and reliability status. Distinguish confirmed facts, estimates, allegations and binding constraints. A contradictory actor statement is not automatically true. Teams maintain an assumptions register and cite numbered evidence.

THE JANUS PROTOCOL | The five-act storyline

Act I: The Evidence Gate

Audit validation and lock initial approve/pilot/restrict/defer/reject conditions. Identify populations, endpoints and stop rules.

Required output: Evidence audit, calculation sheet, thresholds and deployment map.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act II: The Procurement Arena

Actors present competing vendor offers. Rivals file evidence-based red-team questions. Negotiate audit rights, exit, portability and implementation commitments.

Required output: Procurement memo, contract terms, total-cost comparison and response to challenge.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act III: The Mirror Test

Local performance differs from vendor results. Distinguish prevalence effects, data shift and workflow failure using supplied evidence.

Required output: Recomputed metrics, uncertainty statement and deployment revision.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act IV: The Janus Reversal

A safety incident coincides with a product update and apparent subgroup disparity. A media allegation is partly wrong; another mechanism better fits the record.

Required output: Incident reconstruction, pause/continue/rollback decision and correction.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

Act V: The Kill-Switch Hearing

Clinicians, patient, security, vendor and finance actors interrogate the final system. A rival offers a red-team exhibit. Defend deployment, redesign, staged pilot or rejection.

Required output: Safety case, governance charter, fallback plan and monitoring dashboard.

Lock the decision before the next release. Later revisions must identify the changed fact, the old assumption, the new action and its owner. Correcting an error earns adaptation credit; it does not erase the earlier error from the evidence trail.

THE JANUS PROTOCOL | Strategy and rival encounters

Several legitimate routes to victory

- Restricted pilot with prespecified stopping rules.
- Reject the product and improve workflow without AI.
- Require independent audit access before purchasing.
- Separate predictive performance from a measured clinical benefit.

Novelty is not automatically superior. Judges reward feasible alternatives supported by the record. A team can win without accepting a deal, deploying a product or expanding services, provided its alternative meets constraints and withstands challenge.

Hard constraint checks

- Overall accuracy alone cannot establish clinical usefulness.
- Predictive value can change with prevalence.
- Small subgroup samples require explicit uncertainty.
- “Human in the loop” needs a named decision owner and feasible workflow.

A fair confrontation format

Use pods of three or four and rotate counterparties. Publish seeding criteria before competition. An odd field uses a trained actor counterpart. Each team receives one formal rival challenge in designated phases: a numbered exhibit, claimed implication and question. The recipient gets a fixed response window. Judges score the challenge and response independently; no point stealing.

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Consequence engine

Common injects arrive at equal logical checkpoints. Conditional consequences follow disclosed rule categories such as liquidity limits, staffed capacity or validation gaps. Publish the model's constraint relationships and bounded effects in the case instructions, not a secret random multiplier. Staff verify the engine before releasing consequences. Scenario outcomes do not automatically prove decision quality; score controllable analysis and execution.

The drama must be earned

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THE JANUS PROTOCOL | 1,000-point scoring guide

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Criterion / maximum	Evidence for high scores; failure pattern
Clinical validity and safety / 220	Connects metrics to outcomes, alert burden, missed events and meaningful safeguards. Failure pattern: Accuracy treated as sufficient or one error type ignored.
Technical/statistical evidence audit / 200	Correct denominators, provenance, leakage/shift analysis and subgroup uncertainty. Failure pattern: Confused metrics, invented validation or marketing accepted as fact.
Governance, security, privacy and equity / 160	Specific audit rights, data limits, accountable owners and stop authority. Failure pattern: Ethics slogans without enforceable controls.
Workflow, procurement and finance / 140	Real adoption costs, interfaces, staffing, contract protections and alternatives. Failure pattern: Free implementation or no fallback.
Incident response and adaptation / 160	Reconstructs evidence, revises conditions and communicates uncertainty. Failure pattern: Defends the first plan or spreads unverified allegations.
Red-team and stakeholder defense / 70	Precise challenges, direct answers and professional composure. Failure pattern: Unsupported accusations or evasion.
Individual technical mastery / 50	Every student explains an artifact/calculation and cross-role consequence. Failure pattern: One technical specialist shields the team.

Shared performance anchors

Level	Standard
4 — Exceptional	Accurate, complete, case-specific and feasible; resolves conflicts, meets constraints and adapts with explicit reasoning.
3 — Strong	Sound core strategy with minor gaps; supported evidence and credible implementation.
2 — Developing	Partly sound but material assumption, calculation or delivery gaps remain.
1 — Weak	Generic, poorly evidenced or substantially flawed; revisions do not repair the core issue.
0 — Absent/invalid	Required evidence absent, fabricated, irrelevant or incompatible with binding constraints.

THE JANUS PROTOCOL | Scoring worksheet and progression

Criterion	Max	Level	Award
Clinical validity and safety	220	_____	_____
Technical/statistical evidence audit	200	_____	_____
Governance, security, privacy and equity	160	_____	_____
Workflow, procurement and finance	140	_____	_____
Incident response and adaptation	160	_____	_____
Red-team and stakeholder defense	70	_____	_____
Individual technical mastery	50	_____	_____
TOTAL	1000		_____

Individual defense

Ask every student two equivalent short questions: one on work they owned and one on a cross-role dependency. Ask for evidence and reasoning, not obscure trivia. Individual mastery contributes only its listed points; no hidden bonus can change chapter qualification. Record role-specific feedback for development.

Round-specific complexity

Round	Depth, not duration, defines challenge
Regional	One-day five-act case with a concise evidence packet, one principal rival encounter and common core injects.
State	Richer data, tighter constraints, deeper stakeholder conflict and more developed encounter cycles. Expanded logistics appear in the event notice.
National	System-level dependencies and harder uncertainty, with the same published criterion weights. Scheduling approved separately.

What separates this from regular events

A sequence of locked actions creates consequences. Rival encounters test judgment. Technical and clinical evidence must survive execution. The final hearing examines both what the team chose and how it revised. No slide deck can replace the work history.

Safety boundary

All cases are fictional. No patient care, medication administration, real clinical records, unapproved human testing, legal representation or actual investment solicitation. Unsafe proposals affect the relevant scoring criterion; deliberate misconduct follows a separate review process. Do not automatically disqualify a student for an honest analytical mistake.

7 | Judge and director operating manual

The case bible

Required element	Director specification
Truth record	Separate objective truth, actor beliefs, estimates and unresolved uncertainty.
Exhibit register	IDs, units, dates, denominators, reliability and cross-references.
Inject matrix	Release checkpoint, exact script, affected artifacts and equal response window.
Consequence sheet	Constraint formulas, balances, limits, conditional relationships and reviewer checks.
Actor playbook	Permitted concessions, answer boundaries, equivalent pressure and escalation.
Rubric evidence map	Artifacts for each criterion; no repeated scoring or hidden bonuses.
Clarification log	Universal corrections communicated to every team at the same stage.
Audit archive	Locked versions, independent score sheets, calculation checks and appeals.

Staffing estimate for a twelve-team event

Recommended planning baseline: one director; one scenario/control lead; one scorekeeper; three pod moderators; nine judges for three parallel panels; three trained stakeholder actors; two prep proctors; and a safeguarding/accessibility lead. These are estimates, not minimum membership requirements. Reduce field size or stagger sessions if staffing cannot support equivalent supervision.

Build and pilot before launch

- Review every clinical, financial and technical constraint with appropriately knowledgeable adults.
- Test reading load, spreadsheet arithmetic, transition time, role balance and rival encounter scripts with a noncompeting pilot group.
- Score the same exemplar with all judges; investigate major disagreement before the competition.
- Verify each scenario supports at least three feasible approaches rather than one secret preferred solution.
- Prepare equivalent fallback exhibits, actor counterparts and paper tools for technical failure.

8 | Integrity, access and adjudication

Evidence-based integrity

During controlled phases, only supplied materials and authorized tools are allowed. Collect or secure personal devices; archive artifact versions. Do not use AI-detector scores, hidden prompt traps or a judge's intuition as proof. A documented concern triggers evidence preservation, advisor notice, an opportunity to respond and independent review.

Separate deficient work from misconduct

A calculation error or unsafe recommendation normally belongs in the rubric. Fabricated evidence, prohibited assistance, tampering or harassment may trigger published sanctions. Avoid vague global score caps. Graduated responses and disqualification grounds must be published before competition and applied consistently.

Appeals

Proposed window: 30 minutes after provisional results. Appeals may address arithmetic, missing artifacts, unequal materials/timing or a specific rule misapplication. Disagreement with a supported analytic judgment alone is insufficient. An independent lead documents the decision and corrects all affected results before certification.

Access and participant wellbeing

Supply equivalent devices, offline tools, printing and templates. Approve accommodations before the event, preserving assessed skills. Maintain meal and rest breaks. No points for costumes, expensive displays or premium prototypes. Publish travel costs and non-overlapping event schedules. A difficult competition should test judgment, not family wealth or exhaustion.

Rivalry code

- Attack the argument, not the student or school community.
- No sabotage, personal insults, private pressure on actors or invented allegations.
- Cooperation may be a winning strategy; do not punish appropriate medical mutual aid.
- Use consent-based chapter publicity and factual recaps.
- Recognize sportsmanship independently of championship scoring.

9 | Results and adoption checklist

Publish Regional raw scores and qualification. At States, show Regional carryover, State raw score and cumulative total side by side. Name the cumulative State champion clearly. Keep judge working notes, student personal data and sensitive integrity reviews confidential.

Optional recognition

Each event awards one national chapter champion. A proposed Triad Crown may recognize a chapter entering all three through the equally weighted mean of adopted event season scores. It must be approved before the season and cannot change event qualification. Comeback and responsible-leadership honors are separate from advancement points.

Adopt before registration

- Exactly one team of four per school; three events; no second group.
- Top three per Regional pool per event advance; one cumulative champion per State per event advances.
- Confirm the proposed 35% Regional / 65% State formula and publish comparability controls.
- Approve or replace the proposed National formula before the season.
- Regionals: all acts within one day. States: expanded logistics without making duration the headline.
- Publish event blocks so the same delegation can attend without conflicts.
- Pilot cases, actor scripts, consequences, rubrics and hearing capacity.
- Publish tools, safety boundaries, integrity sanctions, appeals and accommodations.

The ambition is a championship students choose for its story, rivals and depth. The credibility comes from understandable rules, meaningful improvement, fair pressure and earned consequences. Launch only when the operating system is as strong as the storyline.